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Falls in Acute Care Settings: A Comprehensive Review of
Risk Factors and Prevention Strategies

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Abstract

Background: Falls remain one of the most common patient safety events in acute care hospitals, with rates ranging from 2.2 to 8.3 falls per 1,000 patient days.

Methods: Systematic search of PubMed, CINAHL, and Cochrane Library for studies published 2018-2024. 42 studies met inclusion criteria.

Results: Key modifiable risk factors: medication effects (OR 2.3), mobility impairment (OR 3.1), environmental hazards (OR 1.7).

Multifactorial interventions showed greatest effectiveness (RR 0.62).

Conclusions: Evidence supports multifactorial fall prevention programs combining medication review, environmental modifications, and patient education.

Case Example - Fictional Patient

Patient: Harold M. Thompson, 72-year-old male

Admission: Elective hip replacement, City General Hospital

Fall Date: Day 3 post-operative

Circumstances: Patient arose from bed at 02:15 AM to use restroom without calling for nurse. Bed alarm activated. Patient found on

floor by nursing assistant. No injuries detected. Morse Fall Scale score: 55 (high risk).

Risk Factors Identified:

- Post-surgical mobility impairment
- Sedative use (oxazepam 15mg at bedtime)
- History of falls (2 previous)
- Cognitive impairment (MMSE 24/30)

Intervention: 1:1 observation, fall precautions, toileting schedule, medication review by pharmacist.

Outcome: No further falls during stay. Discharged with home care.

Evidence-Based Recommendations

1. Implement multifactorial fall prevention programs (Strong evidence)
2. Conduct pharmacist-led medication review within 48 hours
3. Initiate early mobilization protocols
4. Ensure consistent environmental safety measures
5. Provide patient and family education
6. Use validated fall risk assessment tools